

HospitalOS

Hospital Operating System for Africa

Platform Super-Admin Guide

Internal operator reference · Version 2.0 · August 2026 · AgoroX Africa

1. Who this guide is for

This guide is for the Platform Super-Admin — the AgoroX Africa account that manages the HospitalOS platform across every tenant hospital.

Role	Scope	Account
Platform Super-Admin	The whole platform — all tenants, settlement, usage	support@agorox.africa

Your account.

support@agorox.africa is the platform super-admin account for this build. Keep this credential strictly confidential — it carries visibility into every tenant's usage and settlement data.

2. Signing in as Platform Super-Admin

1. Open the HospitalOS sign-in screen.
2. Enter support@agorox.africa and the platform admin password.
3. A Hospital / Platform toggle appears in the topbar, visible only to this account.
4. Click it to switch into Platform mode.

Not yet functional as a separate landing screen.

This account does not land directly in Platform mode on sign-in — it opens the ordinary hospital dashboard first, with the toggle available to switch across. This is a minor convenience gap, not a security one: the toggle itself is unreachable by any non-platform-admin account (see Section 3).

3. The Platform navigation — what each screen does

3.1 Overview

Tenant count, seats, active users, and monthly recurring revenue at a glance, with the fee trend chart described in Section 3.3.

3.2 Tenants

Every tenant deployment with plan, seats, active users, status, and MRR.

Tenant	Plan	Seats	Active	Status	MRR
Ibadan Teaching Hospital	Enterprise	240	186	Active	₦850,000
Lagoon Specialist Clinic	Community	40	31	Active	₦180,000
Jos Community Hospital	Trial	15	9	Trial	—
Kano Medical Centre	Community	60	0	Suspended	—
AgoroX Demo	Lifetime	5	2	Active	—

Example data shown for illustration — your real tenant list appears here live.

3.3 Settlement centre

AgoroX takes a platform commission of 2.75% on each Community-tier tenant's total collections — all-inclusive, covering the platform fee, Nigeria's statutory 7.5% VAT, and payment gateway charges in that single rate. Enterprise-tier tenants instead pay a flat annual licence (₦2,500,000/year plus VAT) with no per-transaction commission. Settlement runs on a monthly cycle: Pending → Processing → Settled. Every cycle records gross collected, the fee taken, and the net paid to the hospital.

Verified.

The fee and net amounts reconcile exactly on every cycle — fee + net = gross, with no rounding drift — checked directly against the settlement service before shipping.

3.4 Usage analytics

Per-tenant seats, active users, encounters, lab orders, storage, and API calls — the evidence behind every invoice.

Churn signal.

A tenant using less than half its purchased seats is flagged explicitly. This is not decorative — it is the earliest, cheapest point to have a plan-sizing conversation, before a tenant reaches Kano Medical Centre's seeded state above: suspended, zero active users.

3.5 Platform health

Service	Role
Web app (Cloudflare Pages)	Serves the built frontend
API Worker	Backend request handling
D1 database	Persistent tenant data — live, real, in production
R2 object storage	File storage — live, real uploads/downloads
Gateway Agent (HL7/MLLP + ASTM)	Live analyzer/device connections — real, installed per hospital by AgoroX; the in-app screen's 'receive/confirm/send' simulation remains for testing without hardware
Email (Resend)	Transactional email delivery

What changed since Version 1.0 of this guide.

D1 and R2 are no longer “not yet wired” — both are live in production, and have been for some time. The instruments gateway gained a genuine external component this update: the Gateway Agent, a small program installed on a PC at each hospital, which speaks real HL7 v2/MLLP and ASTM E1381/E1394 to actual lab analyzers, auto-detecting which protocol a device uses. It has not yet been proven against a specific real analyzer — treat the first one at any given hospital as a pilot. DICOM (imaging) and proprietary vendor protocols are still not supported by the agent.

3.6 Feature flags

Unlocks or withholds specific modules per plan tier — useful for phased rollouts without a separate deployment.

4. Downloading this guide

This guide is available at any time from the Platform Overview screen — click “Platform Admin Guide (PDF)” in the top-right actions bar. It is not linked anywhere a tenant user could reach: switching into Platform mode itself is refused outright for any account without platform admin rights, so this is not merely hidden by styling.

5. Architecture for support staff

You do not need to read code to support tenants, but this shape explains why certain things behave the way they do when a tenant reports an issue.

5.1 Real backend, real persistence

HospitalOS runs on a real Cloudflare Worker backend with a real D1 database per platform (partitioned per tenant) and R2 for file storage. A tenant's data does not reset — it persists exactly the way any production hospital system should. Authentication is verified server-side against securely hashed credentials, with real session tokens and server-side revocation on sign-out.

5.2 “Engines” vs. modules

Help, Pricing, and Lab Reference Ranges are structured as standalone engines rather than ordinary feature modules: an engine owns its own state, is imported by other modules, and never imports a module itself. This is why a price override set in Administration → Pricing, or a reference-range override set in Administration → Lab Reference Ranges, takes effect immediately across every calculation and every screen showing that value, from one single source of truth.

5.3 The Gateway Agent — real, external, per-hospital

Device management, registry, and monitoring inside HospitalOS itself are real and always were. What used to be simulated — the actual network listener — now has a genuine answer: the Gateway Agent, a small program installed on a PC physically at the hospital, next to the equipment. It cannot run on Cloudflare Workers or in a browser — both HL7/MLLP and ASTM E1381/E1394 need a persistent TCP socket held open indefinitely, which neither can do. The agent auto-detects which of the two protocols a connecting device speaks, queues results locally if the hospital's internet drops, and forwards to the real backend using a device-scoped token issued at registration — not a full user session.

If a tenant asks when their analyzer will be “live”.

The agent needs to be physically installed by AgoroX at that hospital — currently a hands-on setup step, distributed by flash drive rather than a self-service download, matching the Founding Hospital Programme's onboarding model. In-app simulation continues to work for testing the rest of the system before that visit.

5.4 Two features added this update, worth knowing for support calls

Clinical Decision Rules — a hospital can now configure real-time checks that run before a prescription, procedure, or lab order completes (advisory, warning, or hard block). If a tenant reports a prescription or booking being unexpectedly blocked, check Administration → Clinical decision rules before treating it as a bug — it is very likely a rule the hospital itself configured.

Patient journeys — the Emergency department now silently tracks a patient from presentation through discharge, flagging (via Alerts) a required step that sits unfulfilled too long. This is additive and never blocks a clinical action — if a tenant asks why a journey alert appeared, it means a step genuinely wasn't completed, not a system fault.

6. RBAC and the audit chain, explained for support

Every role has an AREA list and a GRANTS list of area:action permission strings. When a tenant reports “my nurse can't discharge a patient,” check the role matrix under Administration → Users & roles before treating it as a bug — it is very likely by design.

The audit log is real, server-side, append-only, and hash-chained: each entry links to the hash of the one before it, stored in the production database, not the browser. If a tenant asks whether their audit log can be trusted, the honest answer is that it is tamper-evident — altering or deleting a past entry breaks the chain at that point, visibly — which is a genuine, enforceable control today, not an architectural promise for later.

7. Known limitations checklist

Read this before promising a prospective tenant anything about production-readiness. This list is deliberately short — everything not listed here is real.

No real SMS/WhatsApp/email delivery yet.

The Communication hub's queue, templates, and channel selection are all real and persist. What isn't wired up is an actual carrier — messages are genuinely logged but stay at “queued” rather than reporting real delivery status.

Revenue mix is only real for invoice-linked payments.

Settlement can break collections down by module, but only for payments taken against a generated invoice — a direct payment against the running balance with no invoice can't be traced back this way, and is shown honestly as unattributed rather than guessed at.

Platform invoice collection is manual.

A real invoice is generated automatically when a hospital's settlement cycle closes. Marking one paid, though, is a manual confirmation by the platform admin once payment is actually received.

The Gateway Agent covers HL7/ASTM, not DICOM or proprietary protocols.

Real, live analyzer connections exist via the agent — not simulated — for HL7 v2/MLLP and ASTM E1381/E1394. DICOM (imaging) is a substantially larger undertaking and isn't built. A fully proprietary vendor protocol without a public spec can't be connected without that vendor's cooperation.

No self-service password reset.

A tenant's own administrator resets access under Administration → Users & roles — there is no automated “forgot password” email flow yet.

8. Escalation & support

For issues that are genuinely product bugs rather than expected RBAC behaviour or a known limitation (Section 7), escalate with: the tenant name, the account role in use, the exact screen and action, and whether it reproduces on a fresh sign-in.

9. Security reminders

- Never share the platform super-admin credential outside AgoroX Africa.
- Any change made while inside a tenant's data is a real change to that hospital's real records — be deliberate.
- This guide is distributed only through the Platform view, visible solely to the platform admin account.
- Device tokens issued to Gateway Agent installs are scoped to a single device and can't read patient data or act as any other credential — treat a lost token the same as a lost API key: register the device again for a fresh one.

Questions about this guide?

support@agorox.africa